Nursing Styles

Caregiver Call Off and Open Shift Playbook

How office and on-call staff handle a caregiver call-off, fill the open shift, and keep clients and families informed

Purpose

Every call-off means a client who is counting on us may be left without help. This playbook gives the office team and on-call staff in Pinellas and Sarasota one shared way to take a call-off, decide which visits matter most, find the right replacement, and keep everyone informed, day or night. It works alongside the After Hours Call Workflow, which covers how the AI voice agent takes call-offs at night, on weekends, and on holidays and how it alerts the On-Call Scheduler.

Our goal for every call-off: **no client is left without care and without notice.** Either a qualified replacement is confirmed, or the client and family know what is happening and have agreed to a plan, well before the visit was due to start.

Ground rules

1. People make every staffing decision. Visits are reassigned, unassigned, or cancelled in AxisCare only after a staff member has personally confirmed the change with the replacement caregiver (and, for cancellations, with the client or family). An "Interested" tap in the app, a text reply, or a note from the AI agent is never enough on its own.

2. Client safety comes first. Work the highest priority visit first, not the first call that came in.

3. Minimum necessary information. Record a reason category, never symptoms or diagnoses. Texts carry first names and visit times only. Client details are shared by live voice, in AxisCare, or through secure messaging.

4. Never send someone who is not qualified for the visit, even when time is short. Escalate instead.

5. We do not give clinical advice. Schedulers judge staffing risk from the care plan and AxisCare profile. Questions about a client's health or what care can safely be missed go to the On-Call Nurse or the Director of Nursing.

6. If it is not logged, it did not happen. Every call-off gets a log entry that stays open until it is resolved.

1. Who does what

Role	Responsibilities in this playbook
Scheduler (each office, business hours)	Receives daytime call-offs, finds affected visits, sets priority, offers and fills shifts, updates AxisCare after confirmation, notifies clients and families, keeps the log.
On-Call Scheduler (after hours)	Same duties as the Scheduler outside office hours. Receives call-off alerts from the AI agent under the Urgent staffing ladder in the After Hours Call Workflow.
Office Manager (each office)	First escalation point during business hours. Approves overtime and shift incentives, decides backup plans for unfilled visits, reviews call-off patterns weekly, handles attendance conversations.

Role	Responsibilities in this playbook
On-Call Manager (after hours)	First escalation point after hours for unfilled shifts, caregivers leaving mid-shift, and no-call no-shows. Same decision rights as the Office Manager overnight.
On-Call Nurse / Director of Nursing	Answers any question about a client's care needs, whether a visit can safely be shortened or moved, and what a replacement must be able to do. Scheduling staff do not make these judgments.
Administrator or Owner	Final escalation for any visit still unfilled at the set times, and for decisions that affect agency policy, pay, or client relationships.
AI voice agent (after hours)	Takes and confirms call-offs, records a reason category, and alerts the On-Call Scheduler. It never changes the schedule, never offers shifts, and never calls clients or families.

2. How call-offs come in

Caregivers can reach us several ways. Whatever the channel, a call-off is not "received" until a staff member (or the AI agent after hours) has confirmed the details back with the caregiver and opened a log entry.

Channel	When it is used	What staff do
Phone call to the office	Business hours, Monday to Friday, 9:00 AM to 5:00 PM	Scheduler takes the call using the intake steps below, confirms details, and opens a log entry while still on the phone.
After-hours AI voice agent	Nights, weekends, holidays, and office closures	The AI agent verifies the caregiver, reads back the visit, records the reason category, and alerts the On-Call Scheduler. The On-Call Scheduler acknowledges the alert, reviews the record, and continues from Step 3 (find affected visits). Call the caregiver back if anything is unclear.
Text message	Any time, if the caregiver texts the office or on-call line	Reply with the call-off received template, then call the caregiver to confirm which visit, full call-off or late, and the reason category. Do not discuss client details by text.
AxisCare app message (Availability Team)	Any time	Treat it like a text: acknowledge, call to confirm, then open the log entry. Check the Availability Team inbox at the start of each shift and at least hourly during office hours.
Voicemail or email	Should be rare; caregivers are asked to call	Call the caregiver back as soon as it is heard or read. If the visit starts within 4 hours and the caregiver cannot be reached, treat it as a confirmed call-off and start filling the shift.

Channel	When it is used	What staff do
Told to a coworker or the client	Caregiver mentions it informally	Not accepted as a call-off. Whoever hears it tells the Scheduler or On-Call Scheduler right away, who calls the caregiver to confirm.

Taking the call-off: what to capture

Keep it short, kind, and factual. A caregiver calling off is often already stressed. Thank them for calling, and save any attendance conversation for a manager later.

Field	What to record	Notes
Caregiver	Full name, credential (CNA, HHA, companion), and how identity was checked	Phone number on file, employee ID, or date of birth, as in the After Hours Call Workflow.
Visit or visits affected	Client ID or initials, date, start and end time, office (Pinellas or Sarasota)	Ask whether they need to call off any later visits too, such as the rest of the week.
Type	Full call-off, late arrival (with expected arrival time), or leaving mid-shift	A caregiver leaving mid-shift is always Priority 1.
Reason category	Personal illness, family emergency, transportation, weather or road conditions, child care, or other	Category only. Do not ask about symptoms or diagnoses. If the caregiver volunteers health details, write only the category.
Time received and hours until start	For example: Tuesday 6:10 AM, 1 hour 50 minutes before start	Drives the priority and escalation clock.
Channel and who took it	Phone, AI agent, text, app, voicemail; role of staff member	For the log and pattern review.
Callback number and availability	Where the caregiver can be reached in the next few hours	In case we need a detail about the client, such as a key or door code arrangement.

Read back before you hang up: "Just to confirm, you are not able to work your visit for client [ID or initials] on [day] at [time] because of [category]. Is that right? Thank you for letting us know. We will take it from here. Please keep your phone nearby in case we need you."

Do not tell the caregiver who will replace them or promise that the shift is covered. Do not ask a caregiver who is calling off for illness to find their own replacement.

3. Find every affected visit

1. Open the caregiver's schedule in AxisCare and list every visit they cannot work. Check the next 72 hours at minimum; ask about the rest of the week if the reason may last (for example, a car repair or a family emergency).
2. For each visit, note the client ID, start and end times, service type, and the qualification the visit requires.

3. Check the client's AxisCare profile and care plan summary for the risk factors in Section 4, client preferences (for example, caregiver gender, language, pets, smoking), and the authorized contacts we may notify.
4. Check whether other caregivers are already scheduled at the same home that day (shared or split shifts). A visit with another caregiver already present may be lower risk.
5. Open one log line per affected visit, all linked to the same call-off.
6. Leave the visit assigned to the original caregiver in AxisCare for now, or unassign it to show it as open, following office practice. Either way, do not cancel the visit until a person has agreed to that with the client or family.

4. Decide which visits to fill first

Priority is a staffing judgment based on timing and on what the AxisCare profile and care plan already tell us. It is not a clinical assessment. When you are unsure how much a missed or late visit could affect a client, call the On-Call Nurse or Director of Nursing and let them decide.

Priority	Use when	Response
Priority 1: act now	The visit starts within 4 hours, or The caregiver is leaving mid-shift or did not show up, or The visit starts within 24 hours and the client has two or more of the risk factors below	Drop routine work. Start offering immediately, contact several caregivers at once, and give the manager a heads up right away.
Priority 2: today	The visit starts in 4 to 24 hours and none of the higher triggers apply, or The visit starts within 24 hours and the client has one risk factor	Start offering within 30 minutes of the call-off. Work in the set offer order.
Priority 3: plan ahead	The visit starts more than 24 hours from now	Start offering the same business day. Use the AxisCare open shift broadcast plus direct offers.

Risk factors that move a visit up the list

- Lives alone, or no family or other caregiver will be in the home during the visit.
- Needs hands-on help with transfers, walking, toileting, or bathing, or the care plan notes a two-person or mechanical lift.
- Relies on the visit for medication reminders or assistance at set times.
- Has dementia or memory loss noted in the care plan, especially with wandering or safety notes.
- Relies on the visit for meals or fluids, or the visit is an overnight or 24 hour shift.
- Recently home from the hospital or rehab, or the care team has flagged the client for closer watch.
- The visit starts soon, or the client or family has told us they cannot cover a gap.

Within the same priority level, fill the visit that starts soonest first. If two Priority 1 visits are competing for the same replacement, ask the Office Manager or On-Call Manager to decide.

5. Match the right replacement

A good match keeps the client safe and comfortable and keeps the caregiver set up to succeed. AxisCare can suggest caregivers for an open visit based on preferences, skills, availability, and overtime. Use those suggestions as a starting list, then check each person against the two lists below.

Must have (do not offer the shift without these)

- **Qualification matches the visit.** The caregiver's credential meets or exceeds what the visit requires in AxisCare and the care plan. If you are unsure whether a task needs a CNA or HHA rather than a companion, ask the On-Call Nurse or Director of Nursing before offering.
- **Credentials and compliance are current.** License or certificate, background screening, CPR, required training (for example, dementia care training for clients with dementia), and any other items the agency tracks. Check for expired certification alerts in AxisCare.
- **Truly available.** No overlapping visit, enough travel time from the previous visit, and not already past the agency's limit on consecutive hours.
- **Not excluded by the client.** No "do not send" note on the client profile, and the caregiver has not asked not to work with this client.
- **Overtime approved if needed.** If the shift would put the caregiver into overtime, the Office Manager or On-Call Manager approves it before the offer is made.

Best fit (use these to rank who to call first)

- **Continuity.** Has worked with this client before and the visit went well. Familiar faces matter most for clients with dementia.
- **Client preferences.** Gender, language, pets, smoking, or other preferences on the profile.
- **Location and drive time.** Closest qualified caregivers first. Check realistic drive time at that hour, not just miles. Pinellas and Sarasota caregivers can be offered shifts in the other area only if they have said they are willing.
- **Schedule fit.** Already working nearby that day, or has a gap that the visit fills without overtime.
- **Fairness.** Spread extra shifts across caregivers who want them, rather than always calling the same reliable few.

Qualification quick guide

Visit needs	Who can be offered the shift
Companionship, light housekeeping, meal preparation, errands, and reminders that do not involve hands-on help	Companion / homemaker, HHA, or CNA
Hands-on personal care: bathing, dressing, toileting, transfers, help with walking	HHA or CNA, as set in the care plan
Assistance with medications, or any task the care plan reserves for a certified aide	HHA or CNA with any agency-required training, as set in the care plan. Confirm with the On-Call Nurse if unsure.
Skilled nursing tasks	Not covered by this playbook. Route to the Director of Nursing or On-Call Nurse.

6. Offer the shift in a set order

Offering in a set order is fairer to caregivers, quicker to fill, and easier to explain later. Work down the list below. For Priority 1, contact the first three qualified caregivers at the same time. For Priority 2 and 3, contact one or two at a time.

Step	Who to offer to	How
1	Caregivers who already know this client and are qualified and available without overtime	Phone call first, then the open shift offer text if no answer
2	Caregivers the client or family has asked for, or listed as backups on the profile	Phone call, then text
3	Other qualified caregivers nearby with schedule room and no overtime	Phone call or text, starting with the closest
4	Part-time and as-needed caregivers who have listed availability	Text, then phone call
5	AxisCare open shift broadcast to a selected group of qualified caregivers	Send through AxisCare (app, text, or email). Add key non-PHI details in the comments.
6	Qualified caregivers who would go into overtime	Only after the Office Manager or On-Call Manager approves
7	Escalation options decided by the manager	For example, a shift incentive, a qualified staff member covering, or a plan agreed with the family (see Section 9)

Time limits for each offer

Priority	Wait for a reply before moving on	Contact at once
Priority 1	10 minutes	Up to 3 caregivers
Priority 2	20 minutes	1 or 2 caregivers
Priority 3	2 hours, or by the reply-by time in the offer	1 or 2 direct, plus the AxisCare broadcast

Rules for offering

- Always give a reply-by time in the offer. When it passes, log "no response" and move on. A late "yes" can still be accepted if the shift is open.
- The shift goes to the first qualified caregiver who confirms with you directly. Tell caregivers this up front so no one is surprised.
- An "Interested" or "Accept" tap in the AxisCare app, or a "YES" text, starts the confirmation. It does not finish it. AxisCare can be set to assign a shift automatically when a caregiver accepts it in the app; for this playbook that setting should stay off so a person finalizes every assignment.
- Never include the client's name, address, diagnosis, or care needs in an offer text or broadcast comment. First names, the visit day and time, the credential needed, and a general area are enough. Full details appear in the caregiver's AxisCare app once they are assigned.

- Log every offer: who, when, how, reply-by time, and response. This protects caregivers and the agency if questions come up later.
- Thank caregivers who say no. Declining an extra shift is not a mark against anyone.

7. Confirm with the replacement, then update AxisCare

1. **Talk to the caregiver live** (phone call is best). Confirm the date, start and end time, and that they hold the right credential and can get there on time.
2. **Share the details they need** by voice or in the AxisCare app: client first name, address, entry instructions, care tasks, and any preferences. Remind them the full care plan is in the app.
3. **Update AxisCare only now.** Assign the visit to the replacement caregiver. Add a visit note with the call-off log number, the reason category, and who approved any overtime.
4. **Send the shift confirmation text** and ask the caregiver to reply OK. Check that the visit shows on their AxisCare app.
5. **Close out the other offers.** Send the shift filled message to everyone else you contacted, and withdraw or close the broadcast in AxisCare so no one shows up unexpectedly.
6. **Notify the client or family** (Section 8).
7. **Tell the original caregiver** the shift is covered if they were waiting to hear, and confirm their next scheduled visit.
8. **Update the log** with who filled it, times, and status.

8. Tell the client or family

Clients and families should hear about a change from us, not from a stranger at the door. Only notify authorized contacts listed in AxisCare.

- **When:** as soon as the replacement is confirmed. For Priority 1, or if the visit is still unfilled at its escalation point, call before it is filled to let them know we are working on it.
- **How:** a phone call is preferred, especially for clients with dementia or hearing or vision loss. A text is fine as a follow-up, or on its own for families who have told us they prefer texts.
- **What to say:** the visit day and time, the replacement caregiver's first name, and that they are a qualified member of our team. Share the reason only in general terms ("is not able to make it today"). Never share a caregiver's personal or health reasons.
- **Voicemail:** say only "This is Nursing Styles calling about an upcoming visit. Please call us back at [office number]." No client or caregiver details.
- **If the client declines the replacement:** respect the choice, note it in AxisCare, try another match if time allows, and tell the Office Manager or On-Call Manager. Do not cancel the visit until the client or family agrees and the manager is aware.

Phone script: "Hi, this is [your first name] from Nursing Styles. I'm calling about [client first name]'s visit today at [time]. [Usual caregiver first name] isn't able to make it, so [replacement first name], one of our [CNA / HHA / companion] team members, will be there instead. Is there anything you would like us to share with [him or her] before the visit?"

9. Escalate when a shift is still open

Escalation is not a failure. It brings in someone who can approve overtime or incentives, move staff, or agree a plan with the family. Use the set times below; do not wait for the last minute. After hours, alerts follow the Urgent staffing ladder in the After Hours Call Workflow (On-Call Scheduler, then On-Call Manager at 15 minutes, Office Manager at 30 minutes, and Administrator or Owner at 45 minutes if an alert is not acknowledged).

Priority	Heads up to manager	Escalate if still unfilled	Final escalation
Priority 1	Right away, when the call-off comes in	30 minutes after the call-off, or 2 hours before the visit starts, whichever comes first	1 hour before the visit starts: Administrator or Owner is informed, and the manager calls the family with the backup plan
Priority 2	Not needed	After 2 hours of offers, or 6 hours before the visit starts, whichever comes first	3 hours before the visit starts: Administrator or Owner
Priority 3	Not needed	12:00 PM the business day before the visit	3:00 PM the business day before; family told of any gap by 4:00 PM

What the manager may decide

- Approve overtime or a shift incentive (amount and limits per agency policy).
- Move a caregiver from a lower risk visit, with that client's agreement.
- Have a qualified staff member cover the visit.
- Agree with the client or family on a later start, shorter visit, or family coverage. Any change that could affect care is checked with the On-Call Nurse first.
- If no safe plan exists for a high risk client, the On-Call Nurse and manager decide next steps together, which may include a welfare check with the family. The client is never simply left without a plan.

10. Late arrivals, mid-shift departures, and no-call no-shows

Running late

- Get the expected arrival time. If it is 30 minutes or more past the start, call the client or family, tell them the new time, and ask whether that works. If they cannot wait, treat it as a Priority 1 call-off.
- Log every late arrival, even short ones. They count in the pattern review.

Leaving mid-shift

- Priority 1. Unless it is an emergency, ask the caregiver to stay with the client until a replacement arrives or the manager agrees a plan with the family. Notify the On-Call Manager or Office Manager right away.
- If the caregiver is hurt on the job, make sure they get help first, then follow the agency's injury reporting process.

No-call no-show

A no-call no-show is a caregiver who has not clocked in and has not contacted us by 15 minutes after the visit start. AxisCare late clock-in alerts help catch these early.

1. Call and text the caregiver right away. If they answer and are on their way, treat as running late.
2. If there is no answer within 10 minutes, treat the visit as a Priority 1 open shift and start offering it.
3. Call the client or family to let them know and to check that the client is safe.
4. Notify the Office Manager or On-Call Manager.
5. Keep trying the caregiver. If they cannot be reached at all and there are signs they may be in trouble, the manager decides on any welfare check.
6. Log it as a no-call no-show with the facts only: times of the visit, of each attempt to reach the caregiver, and of any later contact. Attendance follow-up is handled by a manager under agency policy.

11. Documentation

Every call-off is recorded in two places: the call-off and open shift log (Section 14) and AxisCare.

- **AxisCare visit note:** call-off log number, type, reason category, replacement caregiver, who confirmed, and client or family notification. No health details about the caregiver.
- **Log entry:** every field in Section 14, including each offer and each escalation with times.
- **After-hours call-offs:** the On-Call Scheduler resolves the entry from the AI agent's record, so the morning handoff summary shows it as resolved or still open.
- **Status** stays Open until the visit is covered and confirmed, or cancelled with the client or family's agreement. Anything open at 8:30 AM goes to the top of the morning huddle.
- **Keep it factual.** Write what happened and when, not opinions about the caregiver or client.

12. Follow-up and call-off patterns

Most call-offs are honest and unavoidable. Looking at patterns helps us support caregivers and spot problems early, such as a client situation that caregivers find unsafe or a route that is too long.

Weekly review (Office Manager, each office)

- Call-offs and no-call no-shows by caregiver, by day of week, and by shift time.
- Repeated call-offs for the same client or the same type of visit. This can point to a care or safety concern worth raising with the Director of Nursing.
- Reason categories that suggest a fix we can offer, such as transportation or child care.
- Fill rate, average time to fill, number of visits that went uncovered, and how often escalation was needed.
- Caregivers who regularly pick up open shifts, so they can be thanked and recognized.

Caregiver follow-up

- Managers, not schedulers, hold any attendance conversation. Start with a supportive check-in: is something going on that we can help with?
- Thresholds, documentation, and any discipline follow agency HR policy (see Open Questions for Shaun).

- Keep any health information a caregiver shares out of the scheduling log. It belongs only in the HR file, handled under agency policy.

13. Message templates

Placeholders are in [brackets]. Texts include only first names and visit times. Never add a client's full name, address, or anything about their health or care. Send texts from the agency's approved texting or AxisCare messaging, not from a personal phone.

A. Call-off received (text to the caregiver who called off)

> Hi [caregiver first name], this is [your role] at Nursing Styles. We got your message about your [day] [start time] visit. We will call you shortly to confirm. Please keep your phone nearby. Thank you for letting us know.

B. Open shift offer (text to a caregiver)

> Hi [caregiver first name], this is [your role] at Nursing Styles. We have an open [CNA / HHA / companion] shift on [day, date] from [start time] to [end time] in the [general area] area. Are you able to take it? Please reply YES or NO by [reply-by time], or call [office or on-call number]. The shift goes to the first person we confirm with, so it is not yours until we call or text to confirm. Thank you!

For an AxisCare broadcast, use the same wording in the comments. General area means a city or part of the county only.

C. Shift confirmation (text to the replacement caregiver)

> Hi [caregiver first name], thank you! You are confirmed for [day, date], [start time] to [end time]. The visit and care details are now in your AxisCare app. Please check them before you go, and call [office or on-call number] if anything changes. Reply OK to let us know you got this.

D. Shift filled (text to others who were offered)

> Hi [caregiver first name], thank you for getting back to us. The [day] [start time] shift has now been filled. We really appreciate you and will keep you in mind for the next one.

E. Caregiver change notice (text to client or family, after a phone call or when they prefer texts)

> Hi [contact first name], this is [your first name] at Nursing Styles. [Usual caregiver first name] is not able to make the [day] [start time] visit, so [replacement first name] from our team will be there instead. Please call us at [office number] with any questions. Thank you!

F. Still working on coverage (phone script for client or family)

> "Hi, this is [your first name] from Nursing Styles. I'm calling about the visit [today / tomorrow] at [time]. [Usual caregiver first name] isn't able to make it, and we are working right now to arrange another caregiver. I will call you back by [time] with an update. In the meantime, is there anything we should know, or anyone else who can be with [client first name] if we need a little more time?"

G. Unfilled shift escalation (secure message or call to the Office Manager or On-Call Manager)

> Unfilled shift needs your help. Office: [Pinellas / Sarasota]. Log number: [number]. Visit: [day, date], [start time] to [end time], [CNA / HHA / companion]. Priority: [1 / 2 / 3]. Call-off received: [time]. Offers made: [number], no confirmed replacement. Family told: [yes / no]. Overtime or incentive approval needed: [yes / no]. Please call [your role] at [number] now.

If this goes by plain text rather than AxisCare messaging or another secure channel, leave out the client ID and share client details by phone.

14. Call-off and open shift tracking log

Use one line per affected visit. The log can live in a shared, access-controlled spreadsheet or, once set up, in AxisCare. Client details are limited to a client ID or initials.

Main log

Field	What goes here
Log number	Running number per office, for example P-0142 or S-0087
Received	Date and time, channel (phone, AI agent, text, app, voicemail), and role of the person who took it
Caregiver	Name and credential
Visit	Client ID or initials, date, start and end time, service level needed, office
Type and reason	Full call-off, late (with ETA), mid-shift, or no-call no-show; reason category
Priority and hours to start	1, 2, or 3; hours until the visit starts when received
Offers	Number made (details on the offer tracker)
Filled by	Replacement caregiver name and credential, time confirmed live, overtime approved by
AxisCare updated	Time and staff role, after confirmation
Client or family notified	Time, method, who was spoken to (role, for example daughter, authorized contact)
Escalations	Who, when, and decision
Outcome and status	Covered, covered late, shortened with agreement, cancelled with agreement, or uncovered; Open or Resolved; time closed and by whom

Offer tracker (one per open visit)

Time offered	Caregiver	Credential	Method	Reply by	Response	Notes

Sample log layout

Log #	Received	Caregiver	Visit	Type / reason	Pri.	Filled by / time	Family told	Status
P-0142	Tue 6:10 AM, phone, Scheduler	[Name], HHA	Client 2231, Tue 8:00 to 12:00 PM, HHA	Full; transportation	1	[Name], HHA, 6:34 AM	6:40 AM, phone, daughter	Resolved 6:45 AM

Assumptions

- AxisCare is the system of record for clients, caregivers, credentials, visits, and authorized contacts, and staff have the permissions to assign, unassign, and note visits.
- The AxisCare setting that automatically assigns a shift when a caregiver accepts it in the app is turned off, so a person finalizes every assignment.
- The AxisCare Availability Team inbox, open shift broadcasting, and late clock-in alerts are available to the Scheduler and On-Call Scheduler.
- Each office has a Scheduler and Office Manager, and the after-hours roles in the After Hours Call Workflow (On-Call Scheduler, On-Call Manager, On-Call Nurse) are staffed with backups.
- The priority windows, offer response times, 30-minute late threshold, 15-minute no-call no-show definition, and escalation times are starting points for Shaun to adjust.
- Caregiver credential levels and what each may do are set by the care plan and agency policy; the qualification guide in Section 5 is a general guide only.
- Open shift texts may include a general area (city or part of the county) so caregivers can judge the drive, with no street address or client name.

Open questions for Shaun

HR and attendance policy

1. How much notice must caregivers give for a call-off (for example, 4 hours before a shift), and how should late notice be handled?
2. How do we define a no-call no-show (this playbook uses 15 minutes past the start with no contact), and what are the consequences, including when it counts as job abandonment?
3. How many call-offs or late arrivals in what period should trigger a manager conversation, and what does progressive discipline look like?

4. Are call-offs accepted by text and app message, or must caregivers always call? Who is the designated contact for each channel?
5. Are caregivers employees, independent contractors through a registry arrangement, or both? This affects how shifts can be offered, overtime, and attendance follow-up.
6. Can declining offered shifts ever be held against a caregiver, or is it always free to decline?

Operations and scheduling

7. What are the overtime rules: weekly threshold, maximum consecutive hours, and who can approve overtime after hours?
8. Is there a shift incentive for hard-to-fill or last-minute shifts, how much, and who can approve it?
9. May qualified office staff or nurses cover a caregiver visit, and who decides?
10. Which tasks require a CNA or HHA rather than a companion at Nursing Styles, and what extra training (for example, medication assistance or dementia care) is required for which clients?
11. Should Pinellas and Sarasota share caregivers for open shifts, and is there a maximum drive time or mileage policy?
12. Do you agree that AxisCare auto-assign on app acceptance stays off, and which settings should we use for open visit visibility (all open visits or only those sent)?
13. May open shift texts include a city or general area, or only the office region?
14. When a visit cannot be filled, what options may staff offer families (later start, shorter visit, family coverage), and how are billing and cancellation fees handled?
15. Are the priority windows and escalation times in Sections 4 and 9 right for our clients and staffing?
16. Where should the tracking log live (AxisCare or a shared spreadsheet), who can access it, and how long should it be kept?

Quick reference: caregiver call-off

Keep this page by the phone. Full details are in the sections above.

1. Take the call-off

☐ Verify the caregiver. Capture visit(s): client ID or initials, date, times, office.

☐ Type: full, late (ETA), mid-shift. Reason **category only**, no health details.

☐ Read back, thank them, open a log line per visit. Text or app call-offs: acknowledge, then call to confirm.

2. Set priority

☐ **P1:** starts within 4 hours, mid-shift, no-show, or within 24 hours with 2+ risk factors. Heads up to manager now.

☐ **P2:** starts in 4 to 24 hours. **P3:** more than 24 hours out.

☐ Risk factors: lives alone, transfers, medication reminders, dementia, meals or overnight, recent hospital stay, starts soon. Unsure? Call the On-Call Nurse.

3. Match

☐ Must have: right credential, current compliance, truly available, not excluded by client, overtime approved.

☐ Best fit: knows the client, client preferences, closest drive, schedule fit, fairness.

4. Offer in order, with a reply-by time

☐ Knows client > requested or backup > nearby, no overtime > part-time > AxisCare broadcast > overtime (manager OK) > manager options.

☐ Wait: **P1** 10 min (3 at once) | **P2** 20 min | **P3** 2 hours. Log every offer and response.

5. Confirm, then update

☐ Confirm **live** with the replacement. An app tap or YES text is not a confirmation.

☐ **Only then** assign in AxisCare, add the note, send the confirmation text, send "shift filled" to others, close the broadcast.

6. Tell the client or family

☐ Authorized contacts only. Call first; text if they prefer. Replacement's first name and visit time. Voicemail: no details.

7. Escalate if still open

☐ **P1:** 30 min after call-off or 2 hours before start; Administrator or Owner at 1 hour before.

☐ **P2:** after 2 hours of offers or 6 hours before start; Administrator or Owner at 3 hours before.

☐ **P3:** noon the business day before; Administrator or Owner at 3:00 PM; family told by 4:00 PM.

8. Close out

☐ Log: filled by, times, AxisCare updated, family told, escalations, outcome. Resolved or Open.

☐ No-call no-show (15 min, no contact): call and text, P1 after 10 min, check on client, tell manager, facts only.

Never: change AxisCare before a person confirms | send an unqualified caregiver | put client names or health details in a text | give clinical advice.